



ST. PIUS X CATHOLIC HIGH SCHOOL · MODEL UN CONFERENCE 2026

UNITED NATIONS OFFICE ON DRUGS AND CRIME

Combating the Global Opioid Crisis through International Regulation and Rehabilitation Programs

Background Guide

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Introduction

Hello delegates! You will be participating in the United Nations Office on Drugs and Crime (UNODC) committee. Your topic will be addressing the international opioid crisis and combating it with regulations and rehabilitation programs. Every single day, synthetic opioids take hundreds of lives. What was once a medical marvel is now a global emergency. In order to help stop this catastrophe, we expect you, delegates, to intelligently and intentionally come up with ideas and plans of action for your country.

Delegates must research your country's stance on the topic and write a position paper outlining your country's history with opioids, current national policies (or lack thereof), and multiple international solutions the committee can discuss. Other key components to include are past government actions, international frameworks, and the diplomatic strategy to be taken. Your paper must also show a firm understanding of the general global opinion on this opioid crisis.

While natural opioids have been around for thousands of years, in recent times there has been a significant increase in synthetic opioids. This has made drug use more widely available and has led to an economic decline in the places it affects. Because of this recent change, sources must be up to date and reliable to avoid misinformation. Please use a variety of sources and understand your country's effects and stance so discussions are intelligent and informed.

Historical Background

In the past 20 years, the average worldwide drug overdose rate has risen 700 percent. Specifically, the rate rose sharply in the 1990s. Opioids account for 75 percent of these deaths, claiming nearly 400,000 lives (Alpert 3). Many factors have led to this dramatic increase, though no single cause has been widely accepted.

The goal of this committee is to create an international strategy for how to combat the rapid spread of synthetic opioids and mitigate the resulting public health issues, all with the intention of stifling supply, hindering distribution, and overall easing widespread addiction. Possible solutions could include examining past trends and installing early warning systems, tracking current trafficking patterns, and balancing access with regulation. Delegates can acknowledge milestones such as rapid scheduling, expanded access to guidelines and strategies, and intelligence-sharing programs that encourage international cooperation. But while there have been many feats, delegates must also account for limitations like the Dual Crisis Dilemma of illegal drug regulations and the need for pain medication, lack of enforcement, and more. Overall, the body is capable of finding the solution that best addresses the issue at hand while helping as many countries as possible. Delegates should strive for a concrete solution as well as a strong plan of action that can be implemented immediately to fix the spreading global epidemic of synthetic opioids.

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Background Information

Historically, the use of opioids has been around for many years, ranging even as far as 3,400 BC in Ancient Mesopotamia. Here, poppy opium was used for pain relief. But even in these ancient times, there were isolated cases of addiction crises around the world. The Opium Wars between Britain and China are a keen example.

KEY TERM: DUAL CRISIS DILEMMA

The tension between restricting illegal opioid supply and preserving access to opioid-based pain medicine for legitimate medical use.

While these issues with natural opiates were extremely detrimental and important, recently drug addiction has become exponentially worse due to the creation of synthetic opioids. Starting in the 17th century with the separation of compounds, scientists laid the groundwork for drug synthesis by isolating morphine from opium. Less than fifty years later, this evolved into codeine. Consequently, beginning in the 1900s, scientists began to develop semi-synthetic opiates like heroin and oxycodone. Now, in the midst of this crisis, drugs like fentanyl, carfentanil, and tramadol are constructed completely in a lab with little to none of the health benefits that accompany harvested opioids.

In fact, these synthetic opioids have the opposite effect on the body, leading to many health problems. Whereas natural opiates are extracted directly from the poppy and are generally less potent and addictive, synthetic opioids are man-made in a lab and are able to tightly bind to opiate receptors, which makes them highly potent and extremely addictive. Fentanyl itself is around 50 to 100 times more potent than natural morphine. Along with their highly addictive properties, unlike natural opiates, synthetic drugs are very hard to detect on traditional urine drug tests, allowing those who are addicted to go unnoticed and miss out on necessary rehabilitation treatment.

61.3M

People who used opioids worldwide in 2020, up from 26–36M in 2010 (UNODC)

69%

Share of global drug overdose deaths caused by opioids (UNODC, 2022)

This transition to lab manufacturing has also accelerated the crisis. No longer reliant on crop farming and harvest season, the criminals spreading these deadly drugs are able to respond to demand anytime, anywhere with only basic chemical precursors. There has also been a market transition in global illicit activity. Because of the chemical replacement of plant-based drugs, small packages holding large quantities of drugs are harder to detect and harder to regulate. Along with this, because all that is needed is a chemical precursor, drugs are more widely available across borders that were designed only to stop physical illicit activity. This easier access and normalcy surrounding drug abuse has greatly increased the number of overdoses in many countries, turning this into a worldwide crisis.

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Positions of Major Nations

Along with the detrimental effects of the switch from natural to chemical drugs, poverty in different regions and lack of healthcare, combined with the ever-growing black market, has led to a disproportionate effect of opioids on different countries around our world.

United States

In the case of the United States, the fentanyl problem is not just a foreign policy concern but a devastating public health disaster that reaches every single corner of America. The representatives of the country do not stop pressuring other states to take radical measures against illegal supply chains that produce illicit opioids. However, the situation has become so bad that the discussion focuses on saving lives rather than anything else, and there are huge investments in providing people with naloxone, rehab facilities, etc.

Canada

Canada can be summed up as empathetic and pragmatic in its approach. With very difficult overdose statistics, the Canadian government has been a staunch advocate for harm reduction at the international level. Instead of depending on law enforcement alone, Canada calls for the provision of safe consumption sites, safe supplies, and the decriminalization of drugs in small quantities for personal use. Life and de-stigmatization have been at the forefront of the efforts.

United Kingdom

It can be argued that the UK pursues a pragmatic policy that aims at combining tough law enforcement measures with actual help for those who are suffering from addiction problems. When it comes to the international dimension, the representatives of Britain put a lot of emphasis on intelligence sharing and strengthening border controls in order to prevent the distribution of synthetic opioids on the market. Domestically, however, the policy focuses on providing access to treatment.

France

France approaches the crisis through the lens of universal healthcare and human rights. French delegates support global rules to monitor dangerous synthetic chemicals, but draw a hard line on how nations treat their citizens. France firmly opposes overly punitive drug laws that isolate vulnerable people. Instead, they advocate for a system where anyone struggling with substance use can walk into a clinic, get medical care, and access harm-reduction tools like needle exchanges without fear of being treated like a criminal.

Germany

Germany's policy is built on common sense, social support, and practical care. In international meetings, Berlin pushes for smart, coordinated tracking of precursor chemicals across Europe and the UN. But when it comes to actual people, Germany strongly believes that recovery requires stability. They champion a model that includes supervised injection rooms, drug-checking services to prevent accidental poisonings, and easily accessible substitution therapy, arguing that you can't curb supply if you aren't actively taking care of the people affected.

China

China's position is very strict, adopting an entirely law-and-order approach. The country is very proud of its zero-tolerance policy and highlights that it regulated all fentanyl compounds under strict government control to stop any illegal production of the substance. In international forums, Chinese diplomats tend to defend themselves against criticism from Western countries by saying that the problem lies in the high demand and excessive prescription practices in consumer states.

India

India is caught between a rock and a hard place. Being the key producer of cheap drugs for the developing world, India is in favor of banning the sale of these precursors, but it does warn that stringent regulations should not be imposed at a global level. The government of India is known to often remind the international community that many patients in poorer countries are already deprived of even the basic drugs. Any regulations made on a global scale should take care of accessibility to life-saving drugs.

Thailand

Thailand, which has struggled for many years against drug trafficking in its borders, within the region known as the Golden Triangle, understands well the havoc that can be wreaked by this illegal activity. Although Thailand has continued to fight cross-border smuggling networks within its borders, its philosophy at home has changed dramatically. Instead of relying on lengthy prison sentences, Thailand advocates for a health-based approach to substance use problems abroad.

Iran

Iran sits right along one of the heaviest drug transit routes in the world, leaving its security forces in a constant, often violent battle against heavily armed traffickers coming out of Afghanistan. Because of this, Tehran's international message is focused on asking for more global support for its border enforcement. At the same time, the sheer volume of drugs passing through has created a massive domestic addiction problem, forcing Iran to build extensive state-run treatment programs and methadone clinics to support its own population.

Mexico

Mexico's message to the rest of the world is clear: this is a collective problem, and transit states should not shoulder the burden alone. The Mexican people have been adversely affected by cartel-related violence, secret manufacturing operations, and corruption. At the international stage, Mexico takes great pains to counter accusations by other countries because Mexico knows that the consumption-driven demand of those major consumer countries is fueling the drug trade, and the weapons used by the cartels enter the country illegally.

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Timeline of Major Events

- 1946 — United Nations establishes the Commission on Narcotic Drugs.
 - 1961 — United Nations adopts the Single Convention on Narcotic Drugs.
 - 1971 — United Nations adopts the Convention on Psychotropic Substances.
 - 1988 — United Nations adopts the Convention against Illicit Traffic in Narcotic Drugs and Psychotropic Substances.
 - 2009 — WHO publishes Guidelines for the psychosocially assisted pharmacological treatment of opioid dependence.
 - 2016 — UN General Assembly Special Session outcome document, Our joint commitment to effectively addressing and countering the world drug problem.
 - 2019 — CND sets a 2029 review horizon for international commitments.
 - 2020 — WHO and UNODC publish revised International Standards for the Treatment of Drug Use Disorders.
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Key Terms

Opioid. A natural, semisynthetic, or synthetic substance that typically binds to the same cell receptors as opium and produces similar narcotic effects, such as sedation, pain relief, slowed breathing, and euphoria (Merriam-Webster Dictionary).

Opioid overdose. Opioid use leading to death due to the effects of opioids on the part of the brain which regulates breathing (World Health Organization).

Naloxone. A synthetic potent antagonist of narcotic drugs, such as morphine and fentanyl, that is administered especially in the form of its hydrochloride (Merriam-Webster Dictionary).

Fentanyl. A synthetic opioid narcotic analgesic with pharmacological action similar to morphine, administered transdermally as a skin patch or orally and parenterally in the form of its citrate (Merriam-Webster Dictionary).

Nitazenes. Potent opioids increasingly associated with opioid overdoses and deaths (National Library of Medicine).

Methadone. A long-acting opioid medication used to reduce withdrawal symptoms in people addicted to heroin or other narcotic drugs (Drugs.com).

Diversion. Unlawful channelling of regulated pharmaceuticals from legal sources to the illicit marketplace (National Library of Medicine).

Rehabilitation. A structured process that helps people overcome drug or alcohol addiction through medical care, therapy, and long-term recovery support (American Addiction Centers).

Treatment gap. The difference between people who need addiction treatment and those who actually receive it (Institute for Research, Education, & Training in Addictions).

Drug scheduling. Drugs, substances, and certain chemicals used to make drugs, classified into five distinct categories depending upon acceptable medical use and abuse or dependency potential (United States Drug Enforcement Administration).

1961 Single Convention. Combatted drug abuse by coordinated international action (United Nations Office on Drugs and Crime).

1971 Convention. Created an international control system for synthetic and psychoactive drugs (United Nations Office on Drugs and Crime).

1988 Convention. Provides comprehensive measures against drug trafficking, including provisions against money laundering and the diversion of precursor chemicals (United Nations Office on Drugs and Crime).

- CND — UN Commission on Narcotic Drugs
 - INCB — International Narcotics Control Board
 - UNODC — United Nations Office on Drugs and Crime
 - WHO — World Health Organization
 - CDC — Centers for Disease Control and Prevention
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Guiding Questions

1. How can the CND strengthen control on synthetic opioids without reducing access to essential medicines?
2. How can states prevent diversion of pharmaceutical drugs?
3. How can the WHO, UNODC, and INCB share information about emerging opioids?
4. How can states respect sovereignty while combatting international drug crises?
5. How can customs organizations identify drug shipments without disrupting trade?
6. Should states improve access to drugs like methadone?
7. How can countries with limited resources obtain affordable medicines?
8. Should states improve access to naloxone for those likely to witness overdoses?
9. How can governments prevent diversion while ensuring access to morphine and other medicines?
10. Who should finance international treatment and prevention programs?
11. What indicators should be used to measure progress?

Further Research

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- United Nations, "Drugs" — overview of the CND, conventions, and INCB.
- WHO, "Community Management of Opioid Overdose" — guidance on naloxone.
- CDC, "Fentanyl" — data on pharmaceutical versus illicit fentanyl.
- Government of Canada, Federal approach on Canada's Opioid Crisis — public health and enforcement example.
- EUDA, European Drug Report 2025 — European treatment and harm reduction.
- Government of Mexico, national fentanyl prevention campaign.
- UNODC, International standards for the treatment of drug use disorders.

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